



PERSONA

Recert-Clock Riley

EMS Professional Learner · Working
EMT/Paramedic in CE/Recert

VRpatients Brand Foundation · Audit date 2026-05-22 · Pre-questionnaire base draft · Run 5 of 6

Confidence tags **[BEDROCK]** evidence-locked **[CRM-BACKED]** CRM-supported **[ASSUMPTION]** needs human input

1. Identity

A burnt-out, time-starved working EMT or paramedic counting down the 2-year NREMT clock, who'd rather be sleeping than clicking through one more PowerPoint CE module. Riley is the medic in the headset — not the person who decided VRpatients gets bought. **[BEDROCK]**

2. Role Type

User + Influencer. **[BEDROCK]** Never the buyer in VRpatients deals (CRM: 0 of 86 EMS won deals had a line medic as the primary contact; 2 of 54 sampled contacts were paramedics, both secondary). Touches the VR simulation, not the authoring tool — the authoring layer is the training officer's territory (covered under Director Dale).

3. Snapshot

- **Title.** EMT, AEMT, or Paramedic. Settings vary: municipal fire-based EMS, third-service municipal, private ambulance, hospital-based EMS education, college clinical learner-employee. **[ASSUMPTION]** sub-segments not split — see R10.
- **Workforce.** ~282,900 nationally (181,000 EMTs + 101,900 Paramedics, BLS 2024). **[BEDROCK]**
- **Income.** EMT median \$41,340; Paramedic median \$58,410 (BLS 2024). **[BEDROCK]**
- **Schedule.** 12–24 hr shifts dominant; many work multiple jobs. **[BEDROCK]**
- **Burnout.** 52% personal / 49% work-related (NREMT 2022 national survey, n=1,838). **[BEDROCK]**

- **Tech comfort.** Variable. Flight + younger paramedics often early adopters; fire-based and older medics often skeptical of “screens replacing reps.” **[ASSUMPTION]** inferred from category lit; not validated in VRpatients accounts.
- **Segment relevance.** 248 EMS-tagged deals in HubSpot (86 won / 152 lost / ~10 open). EMS is real and material, not a side bet. **[CRM-BACKED]**

4. Goals

- **Functional.** Get the 40–60 NREMT NCCP hours done by March 31 without lapse, without driving to a classroom. **[BEDROCK]**
- **Emotional.** Not feel stupid on the next bad peds, OB, or psych call. **[BEDROCK]**
- **Social.** Be the medic the agency wants on its worst call. Path to flight medic / FP-C / CCP-C credentials. **[BEDROCK]**

5. Pains (ranked, most acute first)

1. **Skill decay on low-frequency / high-acuity calls.** Peds = 5.9% of EMS calls; intubation skill decay documented within 6 months. **[BEDROCK]** Riley's #1 quantitatively documented pain. Peds/OB/geriatrics MVP actively in dev per Sylo (2026-05-22) — partial mitigation incoming.
2. **Burnout + time starvation.** 52% personal burnout; CE competes with shifts, sleep, second jobs. **[BEDROCK]**
3. **Cognitive load of CE compliance.** Tracking National/Local/Individual splits, F1–F5 designations, NREMT audit retention rules. **[BEDROCK]**
4. **Financial pressure.** Out-of-pocket CE bites on a paramedic-median income; 62% of EMTs / 57% of Paramedics report inadequate retirement plans (LEADS). **[BEDROCK]**
5. **Cynicism about CE quality.** “Death by PowerPoint” is industry vernacular; adversarial-compliance relationship with CE. **[ASSUMPTION]** forum-sourced; not validated in VRpatients accounts.
6. **Status pain.** Low pay vs. nursing; 94% of LEADS respondents said EMTs should be paid more. **[BEDROCK]**

6. What They've Tried Before & Why It Failed

- **Vector / DistanceCME / EMSconnect / FOAMfrat / NAEMT RECERT.** CAPCE-accredited online click-through CE. Works for hour count, fails on actual skill maintenance. Reinforces the “death by PowerPoint” cynicism. **[BEDROCK]**
- **Agency mannequin sim (Laerdal SimMan etc.).** Hospital-budget tool that doesn't focus on prehospital problems; airway and IV fidelity gaps. **[ASSUMPTION]** forum-sourced direction-of-effect.
- **Live CE at EMS World Expo / regional conferences.** Once-a-year cluster of hours; doesn't address ongoing decay between conferences. **[BEDROCK]**

Gap VRpatients fills: scenario-based reps for the low-frequency calls that click-through CE can't simulate and the SimMan can't model realistically — if the agency's training officer keeps it deployed and the headset is available.

7. Trigger Events

- **Recert deadline approaching.** Q1 hour-hunt, March 31 cliff. **[ASSUMPTION]** agency-side seasonality not CRM-validated (R12).
- **Lapsed license + reinstatement pain.** **[BEDROCK]**
- **Failed skills check / QA review.** **[BEDROCK]**
- **Protocol/scope change.** E.g., Ohio EMT ondansetron Dec 2025; Virginia “Red Dot” 2022. **[BEDROCK]**
- **Critical incident on shift.** Bad peds code, missed STEMI; self-initiated remediation. **[BEDROCK]**
- **Promotion path.** Flight medic / critical care / training officer track. **[BEDROCK]**
- **NREMT discontinuing ALS psychomotor exam.** Pushes skills-competency documentation onto agencies — tailwind for VRpatients agency motion. **[ASSUMPTION]** not measured in CRM pipeline.

8. Objections (top 3, categorized)

1. **“You don't have pediatrics.”** — Won't work for me. (plus OB and geriatrics.) **[ASSUMPTION]** Web-headlined as the #1 EMS objection; CRM has 0 of 152 EMS losses tagged “Missing features.” Sylo confirms hearing it “quite a few times, especially lately” — peds/OB/geriatrics MVP is in active dev to neutralize.
2. **“Is it CAPCE-accredited? If not, it doesn't count toward my recert.”** — Don't trust the vendor. **[ASSUMPTION]** No public claim of CAPCE provider accreditation found on vrpatients.com or CAPCE directory (audit-time web check, May 2026); internal confirmation needed (R2). If not accredited, Riley personally can't use it for hour count — only the agency can, under local-component approval.
3. **“VR makes me motion sick / I don't have a headset at home / the agency has one for the whole crew.”** — Won't work for me. **[ASSUMPTION]** Deployed-customer incidence unknown (R6, R7).

9. Watering Holes

- **Associations.** NAEMT (owns RECERT online CE), NREMT (credentialing), NAEMSP (medical directors, 1,500+ attendees, 52% physicians at 2026 Annual), NEMSMA (officer/management), NASEMSO (state EMS officials), IAFF/IAFC (fire-based EMS).
- **Conferences.** EMS World Expo (Orlando, Sept 28–Oct 2, 2026 — 7,000+ attendees in 2024; the single most important VRpatients booth event); NAEMSP Annual (Jan 2026 Tampa); Pinnacle EMS Leadership Forum (Jul 2026 San Diego — chief/executive); NAEMSE Educator Symposium (instructor/sim-tech side); JEMS Games at FDIC International.
- **Publications.** JEMS, EMS World, EMS1.com, Journal of Paramedic Practice, Prehospital Emergency Care, Simulation in Healthcare.
- **FOAM channels Riley consumes.** FOAMfrat (\$159.99/yr, CAPCE-accredited — direct competitor for individual CE dollars), FlightBridgeED Podcast (900K+ downloads, listeners in 82 countries), EMCrit (physician-led but heavily consumed by progressive paramedics).
- **Practitioner forum.** EMTLIFE. (Reddit r/ems is larger but deindexed in search; harder to monitor.)

10. Voice of Customer

All quotes forum-sourced; not validated in VRpatients accounts — see R13.

*“The whole recertification process seems like it's going to be more of a pain in the *** than the actual course.”*

EMTLIFE · [ASSUMPTION]

“Hate hate hate trying to navigate the NR system and their seemingly constant changing of how and whats required to recert.”

EMTLIFE · [ASSUMPTION]

“His airway is plastic and unrealistic in terms of airway management.”

EMTLIFE, on mannequin sim · [ASSUMPTION]

“I hope you're good at backflips if you let it lapse.”

EMTLIFE, on lapsed-license reinstatement · [ASSUMPTION]

“Death by PowerPoint.”

Industry vernacular — EMS1, vendor marketing copy · [ASSUMPTION]

Plausible 11pm Google search: “VR paramedic training does it count for CE.”
[ASSUMPTION]

11. Before / After

BEFORE VRpatients works

- Chasing hours toward March 31, clicking through CAPCE-accredited PowerPoint modules at the end of a 12-hour shift, irritated, scoring just-high-enough on the post-test, lapsing one cycle and reinstating in a panic.
- The next peds arrest hits the agency — Riley freezes for 15 seconds, second-guesses the airway, debriefs in private with the medical director.

AFTER VRpatients works

- (VRpatients-deployed agency, training-officer-supported.) Riley puts on a headset between shifts at the station, runs three peds respiratory scenarios with branching outcomes, reviews the AI-graded performance.
- Books a flight-medic prep program with one less “what if I freeze again” worry. CE hours come along for the ride if the agency's local-component approval applies; the real value isn't the hours, it's the reps. [ASSUMPTION] Outcome story plausible but not validated against deployed-account outcomes (R5, R8, R13).

12. Confidence Ledger

[BEDROCK]

- Riley is not the buyer — agency/training officer/chief/program director signs (covered under Director Dale).
- Burnout / income / workforce size / NCCP hour structure / pediatric exposure rate / CE-incumbent landscape all web-validated with named sources.
- VRpatients has a real and material EMS revenue line (248 deals, 86 won).

[CRM-BACKED]

- Buyer titles cluster around Chief / Director / Program Director / Training Officer / Educator.
- Line medics appear only as secondary contacts (n=54 on 25 sampled won deals).
- New-business EMS median ~\$5,400 (n=86); renewals frequently drop to \$1,000 (R4).
- Of 152 EMS losses, only 55 have any reason populated; dominant tags: “No Response” (24), “Limited Budget” (15), “Not Interested at this time” (8).
- “Missing features” = 0; “Takes too much time to build” = 0. 64% of losses have no reason at all (data-hygiene caveat).

[ASSUMPTION]

- “You don't have pediatrics” as the #1 deal-killer.
- CAPCE provider accreditation status.
- “Death by PowerPoint” cynicism in VRpatients accounts.
- March-31-deadline-as-trigger for agency buying.
- Headset-availability and cybersickness incidence in deployed accounts.

Open Questions Routed to Team

Full traceable list in brand/personas/persona-audit-open-questions.md → “Recert-Clock Riley” section. Summary by team:

- **Product + CS.** Does the peds MVP fill Riley's daily skill-rep gap, or only the buyer-objection? (R1); WebGL fallback usage in deployed accounts (R7).
- **Product + Leadership.** CAPCE accreditation status and roadmap (R2).
- **Sales.** The 24 “No Response” EMS losses — what was the last touchpoint? (R3).
- **Sales + CS + Leadership.** \$5K-to-\$1K renewal price erosion — deliberate or discipline gap? (R4).
- **CS.** Do deployed Rileys advocate or merely tolerate? (R5); “Death by PowerPoint” survive contact with VRpatients? (R8).
- **CS + Sales.** Headset-availability constraint real? (R6); Riley sub-segments (R10); Replace EMTLIFE quotes with deployed-account verbatim (R13).
- **CS + Sales + Product.** 2-medic CRM dynamics — gap or non-issue? (R9).
- **Leadership + Marketing.** “Skeptic” persona vs. cynical Riley — same person? (R11).
- **Sales + Marketing.** NREMT March-31 trigger in agency pipeline — real or imported? (R12).

Composite persona. No single named customer was used. Aggregate patterns only. Pre-questionnaire base draft — not for promotion to outputs/final-deliverables/ until the sales / CS / product / leadership questionnaire closes the [ASSUMPTION] gaps.